
Mid-Back and Rib Cage

The crossroads of your upper body — and why
opening it changes everything

NATE RATCLIFF, LMT

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HEALING HANDS BY NATE

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Continuity Member Edition — Month 8

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Published by Healing Hands By Nate

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Introduction

A BETWEEN-SESSIONS MODULE ON THE MOST UNDER-STRETCHED area on almost every body that walks into my office.

If I could only work on one area of the body for the rest of my career, it'd be the mid-back. Everything connects through here. Everything.

I'm not exaggerating. Neck pain? Often a mid-back problem. Shoulder pain? Frequently a mid-back problem. Shallow breathing, low back pain, that "I can't take a full breath" feeling? Mid-back, mid-back, mid-back. After 12+ years on the table I've watched the same pattern play out thousands of times: someone comes in for one of those complaints, we open up the mid-back, and within two or three sessions the complaint quiets down.

The mid-back is the crossroads. And in most adults, it's a closed road.

This module is about opening it back up.

CHAPTER 1

What "the mid-back" actually is

THE MID-BACK IS THE THORACIC SPINE — THE part of your back between your shoulder blades, where your ribs attach. You have 12 thoracic vertebrae, one for each pair of ribs. The whole rib cage is built around this section.

The thoracic spine is supposed to do three things: **rotate** (twist your shoulders to look behind you), **extend** (arch backward), and **flex** (round forward). In a healthy body, all three happen freely throughout the day.

In most adults, especially anyone who sits, drives, or hunches over a phone for hours, the thoracic spine gets stuck in flexion. It rounds forward and stays there. It doesn't rotate well. It doesn't extend at all. The whole crossroads locks down.

Once that happens, anything that should pass through the mid-back has to find another route. That's where the trouble starts.

CHAPTER 2

Why mid-back stiffness causes problems everywhere else

HERE'S THE CASCADE I SEE ON THE TABLE:

Neck pain

When your mid-back can't extend, your neck has to. To look up, to look across, to support your head — your neck does the work your mid-back was supposed to do. The cervical spine wasn't built to be the primary extension joint. It's smaller, less stable, more delicate. Forced to do mid-back work, it gets angry. Persistent neck stiffness in people without an obvious neck injury is usually a downstream signal from a frozen mid-back.

Shoulder problems

Your shoulder blades sit on the back of your rib cage. They glide along it as your arm moves. If the rib cage isn't moving (because the mid-back is locked), the shoulder blades can't move properly either. The shoulder joint compensates by taking on motion it

shouldn't take. Rotator cuff issues, shoulder impingement, that nagging "something's not right" feeling when you reach overhead — frequently downstream of mid-back stiffness.

Shallow breathing

This is the one most people don't connect. Your ribs attach to your thoracic vertebrae. Every breath you take, your ribs lift, your thoracic spine extends slightly, and your lungs expand. If your mid-back is locked, your ribs can't lift fully, and you literally can't take a full breath. Your body adapts by breathing shallower and faster — through the upper chest and shoulders instead of the diaphragm. That's stress breathing. That's nervous-system dysregulation. It feeds back into more tension.

Low back pain

This one's more indirect, but it's there. When your mid-back can't rotate, your low back takes on the rotational load — twisting to reach for something, turning in your seat, looking behind you while backing up. The lumbar spine isn't built for that kind of rotation. Repeated, it strains the discs and the small joints between the vertebrae.

CHAPTER 3

The rib cage connection

PEOPLE HEAR “MID-BACK” AND THINK ABOUT THE spine. The bigger picture is the whole rib cage. Twelve vertebrae, twelve pairs of ribs, plus the sternum in front. It’s one interconnected structure.

When the spine is stuck in flexion, the ribs lock in too. They compress in the back, splay slightly in the front, and lose their normal up-and-down rhythm with breathing. Sit at a desk for years and your whole rib cage takes a shape — narrower, more closed, less mobile.

This affects more than how you look. It affects how deeply you can breathe. How much oxygen your tissues get. How quickly you fatigue under physical work. How well your nervous system recovers from stress (because breath rate is one of the main inputs to nervous-system regulation).

Opening the mid-back means opening the whole rib cage.

CHAPTER 4

What actually helps — three mobility exercises

THESE ARE THE MOVES I'VE FOUND MOST RELIABLY restore mid-back range of motion. None require equipment beyond what's in most houses. All three can be done in 10 minutes or less, daily.

1. Cat-cow

Get on your hands and knees. Wrists under shoulders, knees under hips.

Inhale — drop your belly toward the floor, lift your chest and head. (Cow.) Feel the arch through your whole spine, including the mid-back.

Exhale — round your back toward the ceiling, tuck your chin. (Cat.) Feel the curve through every vertebra.

Move slowly. The point isn't speed — it's getting every segment of your spine to move. Most people start cat-cow and only their low back and neck move; the mid-back stays locked. Pay attention specifically to

that region between your shoulder blades and try to get it to participate.

10 reps. Slow. Twice a day.

2. Open-book stretch

Lie on your side. Knees stacked at 90 degrees. Both arms straight out in front of you, palms together.

Slowly rotate your top arm open like turning the page of a book. Follow it with your eyes. Let your chest open, let the rib cage rotate. Don't force your top shoulder to the floor — just let gravity do what it does.

Hold the end position for 2-3 breaths. Feel the stretch across your chest and through your mid-back.

Return. Repeat 5 times on each side.

This is the single most effective thoracic mobility exercise I know. If you can only commit to one move from this module, make it this one. Two minutes a day. Two weeks. Your mid-back will be noticeably more open.

3. Foam roller extension

If you have a foam roller, lie on the floor with the roller perpendicular to your spine, positioned at mid-back level (right between your shoulder blades).

Arms behind your head, supporting your neck. Slowly extend backward over the roller. Hold the extension for 15-20 seconds, breathing into it.

Then roll the roller slightly up or down on your spine and repeat. Work through the whole mid-back area — usually you'll find one or two segments that feel especially stuck. Spend more time on those.

No foam roller? A tightly rolled bath towel works almost as well.

3-5 positions, 15-20 seconds each. Once a day.

CHAPTER 5

What to expect

MOST PEOPLE WORKING THESE THREE MOVES for two weeks report:

- A feeling of “lightness” through the upper body
- Easier full breaths
- Less neck stiffness
- Less shoulder tension
- A sense of being able to “stand up taller” without forcing it

The reason this works so fast is that the mid-back has been doing nothing for so long that it responds to any input. The first time you actually rotate through your thoracic spine in a year, your whole upper body lights up. Then you do it again the next day. And the next.

Within a week of daily practice, you’ll feel a difference. Within three weeks, other people will notice you stand differently.

CHAPTER 6

When this isn't enough

SOMETIMES MID-BACK STIFFNESS HAS STRUCTURAL causes that mobility work alone won't fix:

- **Scoliosis** — a sideways curve in the spine. Mobility work helps but won't change the curve.
- **Past trauma** — fall on the back, car accident, sports injury. Sometimes the tissue needs hands-on work to release.
- **Severe kyphosis** — extreme forward hunch, often from osteoporosis or long-term postural habits in older adults. Get a physician's input.

If you've been doing this work consistently for a month and feel zero change, come see me. Mid-back issues sometimes need hands-on intervention to break through, especially if there's old scar tissue or a long history of fixed posture. The home practice plus targeted bodywork is what gets most people unstuck.

CHAPTER 7

Your daily 10-minute mid-back open-up

IF YOU DO NOTHING ELSE FROM THIS MODULE, do this every day:

Minute 1-3 — Cat-cow. 10 slow reps. Focus on getting the mid-back to move, not just the low back and neck.

Minute 4-7 — Open-book stretch. 5 reps each side, slow. Breathe into each end position.

Minute 8-10 — Foam roller extension (or rolled-towel substitute). 3-5 positions along the mid-back, 15-20 seconds each.

Ten minutes. Daily. That's the prescription.

CHAPTER 8

Closing

OPEN YOUR MID-BACK AND YOUR NECK, shoulders, and breathing all get better. It's not magic — it's anatomy. The mid-back is the crossroads.

This is the area that gives me the most reliable, most dramatic results in clients who actually do the home work. I've watched chronic shoulder pain disappear over three sessions and a daily open-book stretch. I've watched chronic shallow breathers learn to take full breaths for the first time in years. The mid-back is the keystone — when you put it back in place, the whole arch holds.

Start today. Open book, both sides, five each. Two minutes. Tomorrow do it again.

The companion workbook has a thoracic mobility self-assessment, a 7-day open-up tracker, and three reflection prompts to help you notice the changes as they happen. Use it.

— Nate



This book was crafted by Nate Ratcliff,
LMT and published by Healing Hands By
Nate.